



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : سماح سعيد محمد حسنين
المحافظة : القاهرة
الحى / المدينة : مصر القديمه
رقم البطاقة : 27508140102364 رقم التليفون : 01006090071
العنوان : عمرو بن العاص
العمر (تاريخ الميلاد) : 14-08-1975 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]
قسم العلاج :
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سماح سعيد محمد حسنين National ID: 27508140102364	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

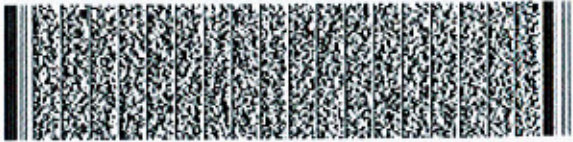
٢٠٢٢/٠٧ ٢٧٥٠٨١٤٠١٠٢٣٦٤

حاصله على بكالوريوس تجارة

أنثى مسلمة متزوجة

محمد عطيه محمد علي

البطاقة سارية حتى ٢٠٢٩/٠٧/٢٨ ١



جمهورية مصر العربية

بطاقة تحقيق الشخصية

سماح

سعيد محمد حسنين يونس

٥١ ش صروبن العاص

مصر القديمة - القاهرة

١٩٧٥/٠٧/١٤

٢٧٥٠٨١٤٠١٠٢٣٦٤

JM1161116

03/04/2026

MDT FORM

Name:	Samah Saeed Mohamed .	MRN:	652
Age:	51 Y	ID:	
Menopausal Sta		Phone:	01006090071
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 8/8 **HER2:** 3 **KI67:** 30%

R

L

Sonomammography:

Mets Workup:

History:

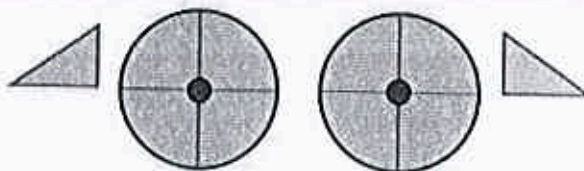
T2 N1
Pt received NACT TXT , carbo*6 , phesigo *5 >>> Rt BCS + ALND >>> PCR
>>> pORT , phesigo
and on tam since 1-2025
Pt reported allergy from tamoxifen and cannot tolerate
for Mdt considering AI +/- zoladex according to hormonal profile

Recommendations:

after DR Hanan consultation >> for AI +/- Zoladex

MDT Outcome & Plan

For Femara + Zoladex



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ هالة عبد العزيز		د/ هالة عبد العزيز			



Patient Name : الأم. م. / م. م. محمد حسين
Patient No. : 27508140102364
Path No. : 2217/24
Sex : Female
Age : 48 Y
Referring Doc. :
Request No. : 13000340
Request Date : 03/07/2024
Verification Date : 21/07/2024
Financial Category : أورام دار السلام - مبادئه
Department : Out Patient
IHC No. :
Registered by : Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical data
Right breast mass

Nature of specimen
Tru cut biopsy, collected at 2/7/2024, 10:30 am

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 70% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 30% nuclear reaction of tumor cells.

Conclusion:

Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 30% of tumor cells.

Verified by :

DR. Menna Nabil

Printing Date : 21/07/2024 10:24:49

Page 1 of 2

Printed By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - محور النيل - مبنى الكاديمية - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr ElQadima - Cairo, Egypt



الأ.م.م.م.

Patient Name : الأستاذة / سماح سعيد محمد حسنين
Patient No. : 27508140102364
Path No. : 2217/24
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13000340
Request Date : 03/07/2024
Verification Date : 14/07/2024
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical data
Right breast mass

Nature of specimen
Tru cut biopsy, collected at 2/7/2024, 10:30 am

Gross examination
Multiple(5) grayish white tissue cores up to 1.3 cm, totally submitted.

Microscopic examination
* Histological type: Invasive duct carcinoma.
* Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
* DCIS: No
* Angiolymphovascular invasion: Couldn't be detected in the received biopsy
* Perineural invasion: No

Diagnosis
Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade II.
For further IHC (ER, PR, HER2, Ki67)

Signature
Dr. Rana Fathy
Dr. Heba Hassan

الجراحة
أ.م.م.م.م.
د. رانيا فathy
د. هبة حسن

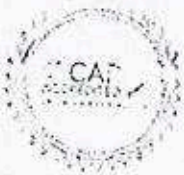
Verified by :
Dr. Heba Hasan

Printing Date : 21/07/2024 10:24:57

Page 1 of 2

Printing By : Mrs. Mayada Hegazy

شرفة رقم (1) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name: Samah Saeed Mohamed Hasanain Yonis	Date : 01/07/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 49 Year(s)
Medical ID: 27508140102364	Pat. ID: 65300

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast pain

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

(ACR C)

- Right Breast:
 - Right LIQ irregular spiculated high density lesion is seen in with adjacent spicules and skin dimpling covering an area measures 41 X 29 mm in CC view
 - It is seen by U/S at 5 o'clock measuring about 2 x 3 cm abutting the skin more than 5 cm from the nipple
 - Pathologically enlarged level I and II axillary LN largest measures 3 X 1.7 cm. (more than three)
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Left Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Right breast malignant looking mass lesions with ipsilateral axillary lymphadenopathy (BIRADS 5) for histopathological examination .
- Normal sonomammography of left breast (BIRADS 1)
- Contrast enhanced spectral mammogram is advisable.

Signature Dr. Mohamed Mohsen

Patient Name: Samah Saeed Mohamed Hasanain Yonis	Date : 28/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 50 Year(s)
Medical ID: 2319/2024	Pat. ID: 65300

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Status follow up compared to last study dated 21/7/2024

1. Chest:

- Right breast LIQ irregular speculated mass lesion measuring 2.8 X 2.2 cm.
- Newly developed ROQ mass 1.6 x1.2 cm
- Right axillary level I pathologically enlarged lymph nodes the largest measures 2.5 x 1.9 cm.
- Few bilateral basal tiny pulmonary nodules the largest 4 mm in left sided.
- Newly developed large subpleural sheets of consolidation extending along upper lobe , middle lobe and anterior segment of right lower lobe Left basal pulmonary fibrotic area with surrounding atelectatic bands and subsequent focal cystic bronchiectatic changes with adjacent air trapping areas, likely sequel of old inflammatory process.
- No sizable hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal non-enhanced CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra-renals.
- underfilling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.

ID: 2769986
 Date: 20/08/2025
 Name: Samah Saeed Mohamed
 Referred by: Prof. Abd Al Wahab Raafat Abd Al Wahab

High Field Dynamic MR-Mammography of Both Breasts

Technique :

- Axial T2 (plate 1).
- Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5,6). Time intensity curves (plate 7).

Findings:

- Bilateral scattered mixed glandular condensation .
- Bilateral background moderate glandular enhancement.
- Right inner central marked postoperative edematous changes and distortion in the form of thickened parenchymal trabeculations , accepted enhanced scar with insitu complex seroma measures about 56x22 mm associated with overlying skin thickening and enhancement ,the kinetic data showed continuous steady rise with late wash out (Type 1b and 2a curves)
- Left scattered foci of punctuate parenchymal enhancement , the kinetic data showed continuous steady rise with o wash out (Type 1 curves).
- Normal skin thickness and contour of left breast.
- No left nipple or skin enhancement .
- Right axillary partial evacuation.
- There is likely non-specific left axillary lymphadenopathy.

OPINION :

Status follow up after right conservative breast surgery for pathologically proven carcinoma ,the current study showed right postoperative changes with insitu complex seroma (**BIRADS 3**).

Left scattered foci of punctuate parenchymal enhancement categorized as **BIRADS 2** based on the morphological and kinetic data.

Bilateral mammary fibroadenosis.

Half-Annual screening is recommended.

Assistant Prof. ENGY ADEL.MD

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category

Suspected Cancer Risk

BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant.	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	



S C A N

Name: Samah Saeed Mohamed

ID: 2769986

Date: 26/08/2025

¹⁸F-FDG PET/CT examination

Clinical Indication:

- Patient with history of right breast cancer underwent surgery and received chemotherapy, radiotherapy and target therapy.

Administered activity: 296 MBq. ¹⁸F-FDG

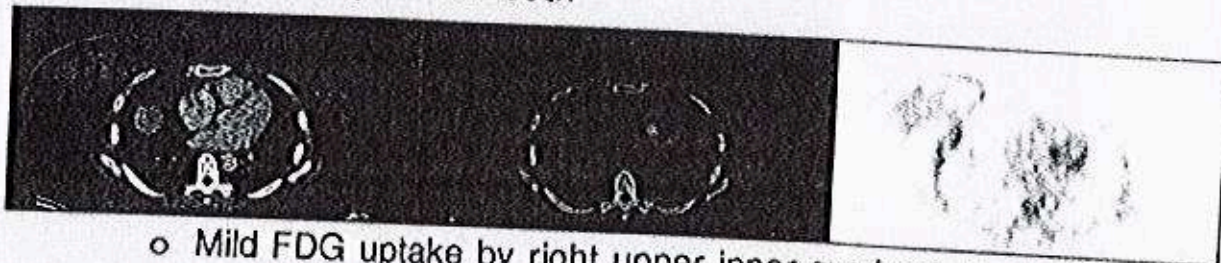
B.G.L.: 112 mg/dL

Examination protocol:

- A Whole body PET/CT study was obtained from skull vertex to the mid thigh using a low dose non contrast CT for attenuation correction
- Diagnostic multislice CT examination after IV contrast administration of the same region.

I. PET/CT Findings:

- Right breast operative bed:



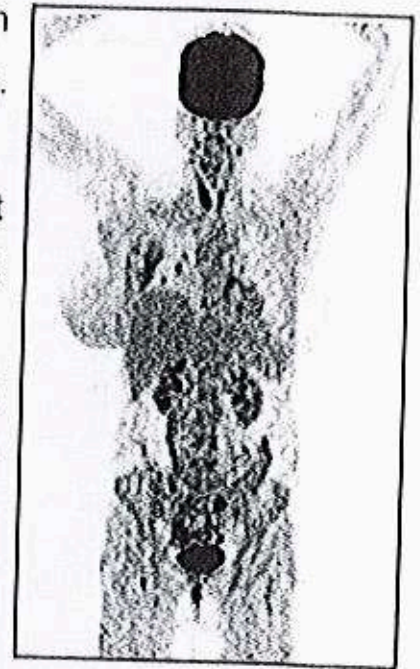
- Mild FDG uptake by right upper inner quadrant predominant fatty lesion, likely traumatic fat necrosis. Adjacent localized photopenic organized collection is noted measures 3.8x2.8 cm.
- The rest of the breast parenchyma is swollen with diffuse mild skin thickening demonstrating low grade FDG uptake.
- No FDG avid masses.
- Left breast: no FDG avid lesions.



S C A N

- Lymph nodes:

- Axillary: left axillary level I lymph nodes with low grade FDG uptake and fatty hilum. SUVmax 1.5 and measures 1.6 cm.
- Cervical: mild increased FDG uptake by left retromandibular lymph node, SUVmax 6.4 and measures 1.2 cm.
- Inguinal: mild FDG uptake within bilateral inguinal lymph nodes with preserved fatty hilum. SUVmax 5.8 and measures 1.3 cm on the right.



- Lungs:

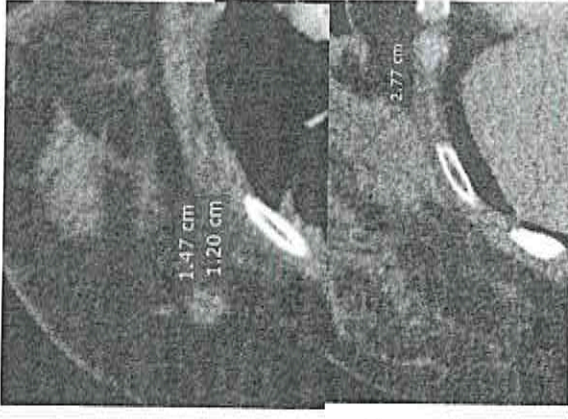
- No FDG avid nodules.
 - FDG activity related to right peripheral anterior consolidative patch, *likely radiation sequelae*.
- Liver: no focal FDG avid lesions. Patent portal vein. No dilated biliary radicles.
 - Pelvic organs, no FDG avid lesions.
 - Bone marrow: no focal FDG avid lytic or sclerotic lesions.
 - Brain: no FDG avid lesions.
 - Thyroid: mild diffuse FDG uptake by thyroid gland, SUVmax 6, *for clinical and thyroid hormonal profile correlation*.
 - FDG activity related to right subcutaneous gluteal soft tissue thickening with fat content, *likely inflammatory*.

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

History: Right breast cancer. No available previous studies for comparison at time of reporting.
I-CHEST:

- Right breast soft tissue nodule measuring 1.5x1.2 cm. A cystic collection with thick wall measuring around 2.7x0.7 cm in axial dimensions is also noted.
- No evidence of sizable pulmonary masses, nodules or cavitations.
- No sizable suspicious supraclavicular, internal mammary, axillary, mediastinal or hilar lymph nodes.
- Middle lobe thick fibrotic bands radiating subpleural.
- Left lower lobe basal fibrotic bands with focal traction bronchiectasis.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size of the heart. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels.



II-ABDOMEN AND PELVIS

- Enlarged liver (18.5 cm) showing diffuse homogenous low parenchymal attenuation likely fatty. No sizable suspicious focal lesions. No dilated biliary radicles.
- Calcular gall bladder.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No masses or back pressure changes. Left upper polar simple cyst measuring 2.2x1.7 cm.
- Normal CT appearance of the spleen, adrenal glands and pancreas.
- Normal CT appearance of the aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no collections.
- No sizable suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Bone window reveals no suspicious lytic or sclerotic lesions.

OPINION :

Known history of right breast cancer, current study reveals:

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- o Right breast soft tissue nodule and cystic collection for sonomammography correlation.
- o No sizable pulmonary, abdominopelvic or osseous metastatic deposits.

Shohenda. M. Taha, MD

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Status post right CBS , routine follow up study.
Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
-

LIQ architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.

- **Ultrasound demonstrated LIQ area of parenchymal disruption underneath the operative scar with seroma measuring (4x 2 cm).**
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

- **Right breast (BIRADS 2): Post-Therapeutic sequelae with seroma formation.**
- **Left breast (BIRADS 1): Normal study**

Recommendation: -

- **For annual follow up.**
- Prof.MONA ZAKY

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-18
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC

Findings:

- No cerebral, cerebellar or brain stem areas of altered MR signal.
- Normal size and configuration of the supra and infra tentorial ventricular system.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

➤ Normal MRI of the brain.

Dr.Assem Ahmed

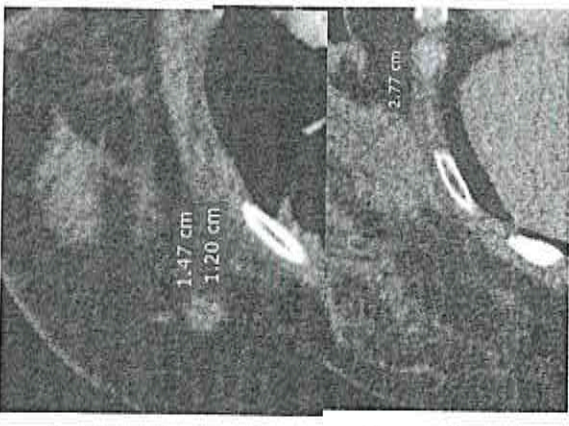
Dr.Assem Ahmed

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

History: Right breast cancer. No available previous studies for comparison at time of reporting.
I-CHEST:

- Right breast soft tissue nodule measuring 1.5x1.2 cm. A cystic collection with thick wall measuring around 2.7x0.7 cm in axial dimensions is also noted.
- No evidence of sizable pulmonary masses, nodules or cavitations.
- No sizable suspicious supraclavicular, internal mammary, axillary, mediastinal or hilar lymph nodes.
- Middle lobe thick fibrotic bands radiating subpleural.
- Left lower lobe basal fibrotic bands with focal traction bronchiectasis.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size of the heart. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels.



II-ABDOMEN AND PELVIS

- Enlarged liver (18.5 cm) showing diffuse homogenous low parenchymal attenuation likely fatty. No sizable suspicious focal lesions. No dilated biliary radicles.
- Calcular gall bladder.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No masses or back pressure changes. Left upper polar simple cyst measuring 2.2x1.7 cm.
- Normal CT appearance of the spleen, adrenal glands and pancreas.
- Normal CT appearance of the aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no collections.
- No sizable suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Bone window reveals no suspicious lytic or sclerotic lesions.

OPINION :

Known history of right breast cancer, current study reveals:

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- o Right breast soft tissue nodule and cystic collection for sonomammography correlation.
- o No sizable pulmonary, abdominopelvic or osseous metastatic deposits.

Shohenda. M. Taha, MD

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Status post right CBS , routine follow up study.
Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
-

LIQ architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.

- **Ultrasound demonstrated LIQ area of parenchymal disruption underneath the operative scar with seroma measuring (4x 2 cm).**
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

- **Right breast (BIRADS 2): Post-Therapeutic sequelae with seroma formation.**
- **Left breast (BIRADS 1): Normal study**

Recommendation: -

- **For annual follow up.**

Prof.MONA ZAKY

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-18
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC

Findings:

- No cerebral, cerebellar or brain stem areas of altered MR signal.
- Normal size and configuration of the supra and infra tentorial ventricular system.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

➤ Normal MRI of the brain.

Dr.Assem Ahmed

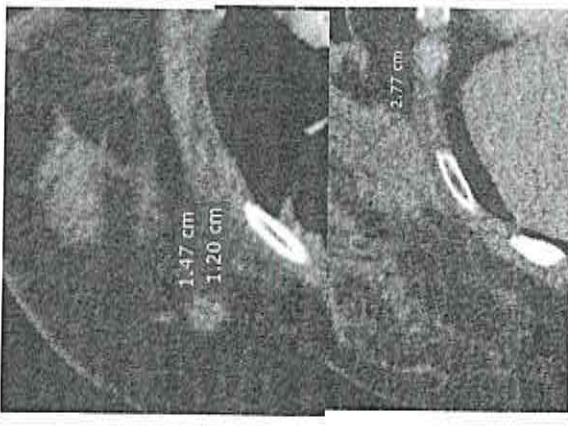
Dr.Assem Ahmed

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

History: Right breast cancer. No available previous studies for comparison at time of reporting.
I-CHEST:

- Right breast soft tissue nodule measuring 1.5x1.2 cm. A cystic collection with thick wall measuring around 2.7x0.7 cm in axial dimensions is also noted.
- No evidence of sizable pulmonary masses, nodules or cavitations.
- No sizable suspicious supraclavicular, internal mammary, axillary, mediastinal or hilar lymph nodes.
- Middle lobe thick fibrotic bands radiating subpleural.
- Left lower lobe basal fibrotic bands with focal traction bronchiectasis.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size of the heart. No pericardial effusion detected.
- Normal enhanced appearance of the thoracic aorta and the great vessels.



II-ABDOMEN AND PELVIS

- Enlarged liver (18.5 cm) showing diffuse homogenous low parenchymal attenuation likely fatty. No sizable suspicious focal lesions. No dilated biliary radicles.
- Calcular gall bladder.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No masses or back pressure changes. Left upper polar simple cyst measuring 2.2x1.7 cm.
- Normal CT appearance of the spleen, adrenal glands and pancreas.
- Normal CT appearance of the aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no collections.
- No sizable suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Bone window reveals no suspicious lytic or sclerotic lesions.

OPINION :

Known history of right breast cancer, current study reveals:

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- o Right breast soft tissue nodule and cystic collection for sonomammography correlation.
- o No sizable pulmonary, abdominopelvic or osseous metastatic deposits.

Shohenda. M. Taha, MD

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-30
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: Status post right CBS , routine follow up study.
Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Thickened skin and coarsen trabeculae.
-

LIQ architectural distortion with no speculations, masses, asymmetry, or suspicious microcalcifications.

- **Ultrasound demonstrated LIQ area of parenchymal disruption underneath the operative scar with seroma measuring (4x 2 cm).**
- Thickened skin and coarsen trabeculae.
- Clear right axilla. No pathological infra or supra-clavicular lymph nodes

Left breast.

- Scattered fibroglandular densities. (ACR b)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin, nipple/areola.
- Non-specific axillary LNs.

Conclusion: -

- **Right breast (BIRADS 2): Post-Therapeutic sequelae with seroma formation.**
- **Left breast (BIRADS 1): Normal study**

Recommendation: -

- **For annual follow up.**

Prof.MONA ZAKY

Patient's Name: Samah Said Mohammed Hasanen
Date: 2026-08-18
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC

Findings:

- No cerebral, cerebellar or brain stem areas of altered MR signal.
- Normal size and configuration of the supra and infra tentorial ventricular system.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.

OPINION:

➤ Normal MRI of the brain.

Dr.Assem Ahmed

Dr.Assem Ahmed